NCCP 00505 · VERSION 5

Your treatment, explained

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a. · NCCP 00505 v5 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Capecitabine + temozolomide (CAPTEM)

28 days: capecitabine D1–14, temozolomide D10–14, then rest D15–28

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Call urgently for new cough/breathlessness, bleeding or yellow skin/eyes. Chest pain or severe breathlessness needs emergency help. Do not restart held treatment without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · smpc · temozolomide-pil

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

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UNDERSTAND YOUR TREATMENT

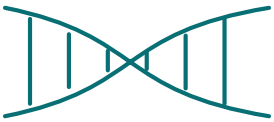
How the two medicines work

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

CHEMOTHERAPY TABLET

Capecitabine

Xeloda / generic



Converted to fluorouracil inside the body

Capecitabine is changed into fluorouracil, which interferes with the processes cells use to make and use their genetic instructions. This can damage cancer cells, but healthy bowel, mouth, skin and blood-forming cells can also be affected.

Tablet taken → Converted to fluorouracil → Cell growth disrupted

What to watch for

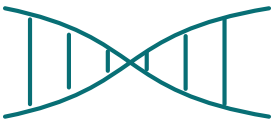
Stop and call for significant diarrhoea, mouth or hand-foot symptoms; chest pain needs emergency assessment.

Sources: smpc

CHEMOTHERAPY CAPSULE

Temozolomide

Temodal / generic



Damages DNA in cells

Temozolomide produces DNA damage that can prevent cancer cells from continuing to grow. Blood-forming cells and other healthy tissues can also be affected, so nausea prevention, blood counts, infection awareness and liver tests are important.

Capsule taken → DNA damaged → Cell growth disrupted

What to watch for

Take the prescribed anti-sickness plan on temozolomide days; report fever, bleeding, new breathing symptoms or liver symptoms.

Sources: temozolomide-pil · nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Energy / headache

CAPTEM

Fatigue or headache can affect daily tasks. Report marked new symptoms; sudden confusion, collapse or a seizure needs emergency help.

3 Heart / chest

Capecitabine

Chest pain, pressure or severe breathlessness needs emergency assessment. Tell staff you are taking capecitabine.

5 Lungs / infection

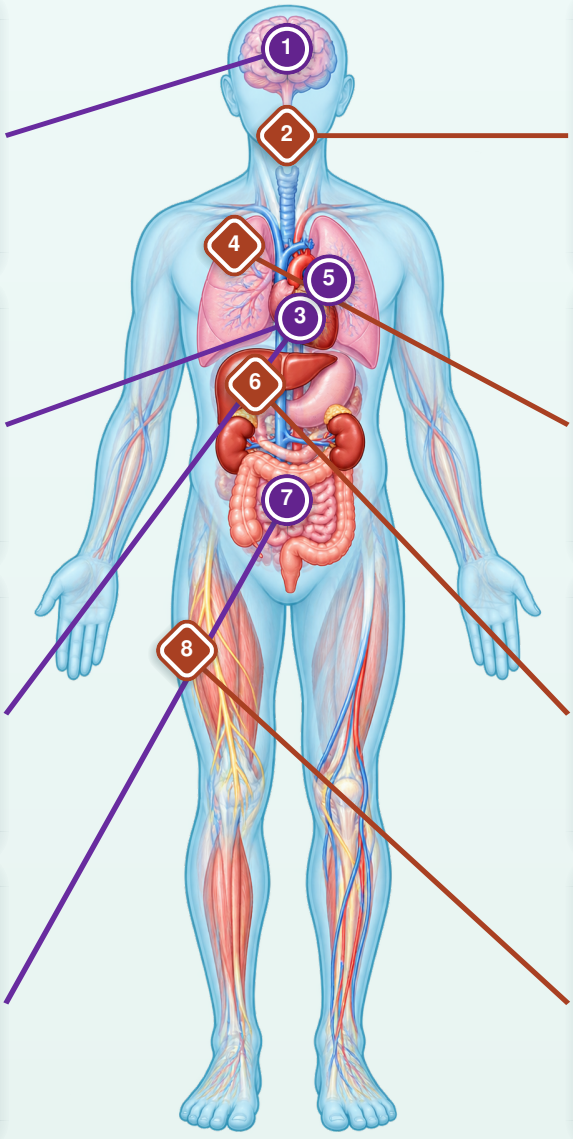
Temozolomide

New cough, breathlessness or fever needs prompt review. The team considers infection, including PJP, and any prevention plan.

7 Bowel / nausea / fluids

CAPTEM

Diarrhoea or vomiting can cause dehydration. Take prescribed anti-sickness medicine and call for persistent symptoms or reduced urine.



2 Mouth / eating

Capecitabine

Mouth pain or ulcers can prevent eating and drinking. Stop capecitabine and contact the team if symptoms are significant.

4 Blood / infection

CAPTEM

Counts can fall, including after capsule days finish. Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent advice.

6 Liver

CAPTEM

Liver injury can develop during or after treatment. Attend tests after each cycle and report yellow skin/ eyes or dark urine.

8 Hands / feet / skin

Capecitabine

Painful redness, swelling or blisters can limit walking or hand use. Stop capecitabine and call when symptoms are significant.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Two medicines, different treatment days

DIFFERENT TABLET DAYS

CAPTEM · six planned 28-day cycles

D1–9 | D10–14 | D15–28

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

The two medicines overlap only on Days 10–14. Six cycles are planned; dose changes, holds or stopping depend on your team's assessment.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Safety tests before, between and after CAPTEM cycles

Six cycles are planned in NCCP; the team may change or stop treatment for progression or toxicity. Do not substitute a fixed scan/marker timetable from another NET treatment.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00505 v5 · current Irish CAPTEM regimen · EMA Xeloda product information and patient leaflet · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Capecitabine + temozolomide (CAPTEM): treatment and checks

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a.

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment / reviews	Between visits	Response review	After a pause or stop
Blood count / kidneys / liver NCCP 00505	FBC, renal and liver profile	Before each cycle; liver tests also after each cycle	Earlier assessment for infection, bleeding, dehydration or symptoms	Review recovery before another cycle	Liver effects may arise weeks after the final dose
DPD / hepatitis B / INR NCCP 00505	DPD before capecitabine unless already tested; hepatitis B surface antigen and core antibody	INR if indicated, especially with warfarin	Watch for unusually severe early toxicity	Specialist review of any DPD-related risk or interacting medicine	Follow specialist and anticoagulation plans
Symptoms / oral medicines NCCP 00505	Record usual bowel pattern, intake, mouth, skin and activity	Review adherence, worst symptoms and any doses held	Nausea plan on D10–14; symptom calls on all days	Confirm both prescriptions and the next start date	Follow persistent symptoms and return unused medicine
NET response / duration NCCP 00505	Confirm pancreatic NET, goal and six-cycle plan	Scans / hormone markers according to the consultant plan	Track symptoms; report change without waiting for imaging	Balance response, toxicity and patient goals	Discuss follow-up or next treatment after the intended course

What each check is for

1

CAPTEM · six planned 28-day cycles

Different tablet days

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00505

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Capecitabine twice daily	Days 1–14	With food or within 30 minutes after food.
Temozolomide once daily	Days 10–14	Fasting; follow the separate anti-sickness plan.
No CAPTEM tablets / capsules	Days 15–28	Recovery, liver tests and next-cycle assessment.
Symptoms and safety	Days 1–28	Serious symptoms can occur during rest days too.

Two calendars in one cycle: Capecitabine starts on Day 1; temozolomide starts on Day 10. The overlap lasts five days, then both stop for Days 15–28.

Rest days still need symptom awareness: Blood counts and other effects may worsen after the tablets finish. Continue the emergency plan throughout the cycle.

Delayed liver effects: Keep the arranged liver tests after each cycle and any follow-up after the last dose; do not assume all risks end on Day 14.

> **Why these bands stay broad:** Treatment timing is shown for orientation. Symptoms can develop on any day; no symptom band predicts an individual toxicity peak. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00505 v5 · current Irish CAPTEM regimen ↗

> **There are no guaranteed "safe days".** Follow your oncology unit's emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Complete timing and symptom notes

Day 1–9: Capecitabine only

Take the prescribed morning and evening doses with food or within 30 minutes afterwards. Check your calendar and report bowel, mouth or hand-foot symptoms early.

Day 10–14: Both medicines; different food instructions

Continue capecitabine twice daily. Add the once-daily temozolomide dose fasting, with the prescribed anti-sickness plan. Ask the pharmacist to write practical dose and meal times; do not assume both medicines are taken together.

Day 15–28: Planned rest from both anticancer medicines

Take neither CAPTEM medicine unless the team has changed your written plan. Symptoms and low blood counts may continue. Complete liver tests after the cycle and blood tests before the next. Restart only when authorised.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dose and meal timing

The source starting schedule is capecitabine 750 mg/m² twice daily on Days 1–14 and temozolomide 200 mg/m² once daily on Days 10–14 of a 28-day cycle. Your own label gives the actual tablet/capsule numbers. Take capecitabine with food or within 30 minutes afterwards; take temozolomide fasting according to the pharmacy instructions. Swallow both whole; do not crush, cut or open them.

STEP 2

Missed or vomited doses

Do not double capecitabine or take a replacement dose after vomiting. If you miss temozolomide, follow the pharmacy’s written instructions or call before taking it late. If you vomit after temozolomide, do not take a second dose that day. Never replace doses omitted because of toxicity.

STEP 3

Support medicines

Have the Days 10–14 anti-sickness prescription ready before Day 10. Ask how it fits around fasting temozolomide and meals for capecitabine. Follow the local rescue and constipation/diarrhoea plan.

STEP 4

Medicine interactions

Bring a complete medicine list. Warfarin may need extra INR checks. Tell the team about seizure medicines and recent brivudine or related antiviral treatment; dangerous interactions require specialist review.

STEP 5

Storage and handling

Keep each medicine in its own labelled original packaging and away from children. Do not cut or crush capecitabine or open temozolomide capsules. If swallowing is difficult, contact pharmacy before treatment. Return unused medicine as advised.

STEP 6

Pregnancy, breastfeeding and fertility

These medicines can harm a pregnancy and may affect fertility. Discuss contraception for patients and partners, the required duration after the last dose, fertility preservation and breastfeeding before starting; follow the medicine-specific written plan.

STEP 7

Review appointments

Before starting: blood count, kidney/liver tests, hepatitis B screening and DPD testing unless already tested. Blood count, kidney and liver tests before every cycle; liver tests also after each cycle. INR if indicated with warfarin. Liver injury can appear weeks after the last dose. Scans and hormone markers follow the consultant plan.

Sources: NCCP 00505 v5 · current Irish CAPTEM regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Separate nausea-prevention plan

Capecitabine is minimal-to-low emetogenic, while temozolomide is moderate-to-high emetogenic in this oral schedule. The team must prescribe an oral anti-sickness plan for Days 10–14 and any rescue treatment; this is not a single low-risk infusion plan.

2 Mouth, hands and feet

Use the unit's mouth and skin care. Significant pain, ulcers or hand-foot symptoms may require capecitabine interruption; do not keep taking tablets just to finish the fortnight.

3 Diarrhoea and fluids

Ask for written diarrhoea instructions and the point at which to stop capecitabine and call. Persistent vomiting, severe diarrhoea, dizziness or inability to drink needs urgent advice.

4 Infection and PJP prevention

The team assesses whether PJP prevention is needed under local policy, especially with steroid exposure or other risk factors. CAPTEM is not the 42-day temozolomide-plus-radiotherapy schedule; do not borrow that schedule's automatic prevention plan.

Sources: nccp · smpc · temozolomide-pil

TREATMENT REFERENCE · 01

Your CAPTEM treatment

Two chemotherapy medicines with carefully separated instructions.

Why CAPTEM?

CAPTEM combines two oral chemotherapy medicines to help control an advanced pancreatic NET. The team discusses the intended benefit, side effects and alternatives. NCCP identifies this pancreatic NET use as outside the medicines' licences, requiring the local consent process.

Sources: nccp · smpc · temozolomide-pil

What the medicines do

Capecitabine is changed into fluorouracil, which interferes with the processes cells use to make and use their genetic instructions. This can damage cancer cells, but healthy bowel, mouth, skin and blood-forming cells can also be affected. Temozolomide produces DNA damage that can prevent cancer cells from continuing to grow. Blood-forming cells and other healthy tissues can also be affected, so nausea prevention, blood counts, infection awareness and liver tests are important.

Sources: nccp · smpc · temozolomide-pil

Schedule and planned duration

Each cycle lasts 28 days: capecitabine twice daily on Days 1–14; add temozolomide once daily on Days 10–14; take neither anticancer medicine on Days 15–28. NCCP plans six cycles unless progression or unacceptable toxicity changes the plan.

Sources: nccp · smpc · temozolomide-pil

Evidence and uncertainty

Adding capecitabine improved the time before disease progression in the trial. The final overall-survival comparison was not statistically significant. Progression-free survival is not a personal survival forecast. The study population and trial treatment duration do not replace the Irish six-cycle regimen, consent process or individual decision. Routine MGMT testing is not mandated by this source.

Sources: e2211

TREATMENT REFERENCE · 02

Your tablet, capsule and support plan

Keep a written dose and meal-time calendar.

Dose and meal timing

The source starting schedule is capecitabine 750 mg/m² twice daily on Days 1–14 and temozolomide 200 mg/m² once daily on Days 10–14 of a 28-day cycle. Your own label gives the actual tablet/capsule numbers. Take capecitabine with food or within 30 minutes afterwards; take temozolomide fasting according to the pharmacy instructions. Swallow both whole; do not crush, cut or open them.

Sources: nccp · smpc · temozolomide-pil

Missed or vomited doses

Do not double capecitabine or take a replacement dose after vomiting. If you miss temozolomide, follow the pharmacy's written instructions or call before taking it late. If you vomit after temozolomide, do not take a second dose that day. Never replace doses omitted because of toxicity.

Sources: nccp · smpc · temozolomide-pil

Support medicines

Have the Days 10–14 anti-sickness prescription ready before Day 10. Ask how it fits around fasting temozolomide and meals for capecitabine. Follow the local rescue and constipation/diarrhoea plan.

Sources: nccp · smpc · temozolomide-pil

Medicine interactions

Bring a complete medicine list. Warfarin may need extra INR checks. Tell the team about seizure medicines and recent brivudine or related antiviral treatment; dangerous interactions require specialist review.

Sources: nccp · smpc · temozolomide-pil

Separate nausea-prevention plan

Capecitabine is minimal-to-low emetogenic, while temozolomide is moderate-to-high emetogenic in this oral schedule. The team must prescribe an oral anti-sickness plan for Days 10–14 and any rescue treatment; this is not a single low-risk infusion plan.

Sources: nccp · smpc · temozolomide-pil

Mouth, hands and feet

Use the unit’s mouth and skin care. Significant pain, ulcers or hand-foot symptoms may require capecitabine interruption; do not keep taking tablets just to finish the fortnight.

Sources: nccp · smpc · temozolomide-pil

Diarrhoea and fluids

Ask for written diarrhoea instructions and the point at which to stop capecitabine and call. Persistent vomiting, severe diarrhoea, dizziness or inability to drink needs urgent advice.

Sources: nccp · smpc · temozolomide-pil

Infection and PJP prevention

The team assesses whether PJP prevention is needed under local policy, especially with steroid exposure or other risk factors. CAPTEM is not the 42-day temozolomide-plus-radiotherapy schedule; do not borrow that schedule’s automatic prevention plan.

Sources: nccp · smpc · temozolomide-pil

TREATMENT REFERENCE · 03

Tests, urgent symptoms and follow-up

Some effects appear after treatment days have ended.

Before, during and after cycles

Before starting: blood count, kidney/liver tests, hepatitis B screening and DPD testing unless already tested. Blood count, kidney and liver tests before every cycle; liver tests also after each cycle. INR if indicated with warfarin. Liver injury can appear weeks after the last dose. Scans and hormone markers follow the consultant plan.

Sources: nccp · smpc · temozolomide-pil

When to stop and call

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Call urgently for new cough/breathlessness, bleeding or yellow skin/eyes. Chest pain or severe breathlessness needs emergency help. Do not restart held treatment without instructions.

Sources: nccp · smpc · temozolomide-pil

Unusually severe early effects

DPD helps break down capecitabine’s active drug. DPD testing reduces risk but does not remove it. Severe early diarrhoea, mouth soreness, infection or neurological symptoms needs immediate team assessment; do not continue waiting for the next cycle.

Sources: nccp · smpc · temozolomide-pil

Storage and handling

Keep each medicine in its own labelled original packaging and away from children. Do not cut or crush capecitabine or open temozolomide capsules. If swallowing is difficult, contact pharmacy before treatment. Return unused medicine as advised.

Sources: nccp · smpc · temozolomide-pil

Pregnancy, breastfeeding and fertility

These medicines can harm a pregnancy and may affect fertility. Discuss contraception for patients and partners, the required duration after the last dose, fertility preservation and breastfeeding before starting; follow the medicine-specific written plan.

Sources: nccp · smpc · temozolomide-pil

TREATMENT REFERENCE · 04

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour (NET) starts in cells that receive signals and can release hormones. The original organ is the primary site. A pancreatic NET is different from the more common pancreatic adenocarcinoma. Stage describes how far the cancer has spread; the stage system depends on its original site. A tumour in the liver may be a NET that spread there from elsewhere.

Sources: nccn-patient

Differentiation, grade and Ki-67

Differentiation describes how the cells look under the microscope. Grade describes their growth activity. Ki-67 is a laboratory measure of the proportion of tumour cells showing a marker of cell growth and helps assess grade in relevant NETs. It is not the percentage of your body affected and is not a survival percentage. Ask for the exact pathology wording, grade and Ki-67 result, including the sample date.

Sources: nccn-patient

Grade is different from stage

A low-grade NET can have spread, and a high-grade tumour is not automatically stage 4. Well-differentiated grade 3 NET and poorly differentiated neuroendocrine carcinoma are different diagnoses with different treatment choices. These medicine guides must not be applied to another diagnosis simply because its name contains neuroendocrine.

Sources: nccn-patient

Functioning and nonfunctioning

A functioning NET causes a hormone-related syndrome. A nonfunctioning NET does not cause a recognisable excess-hormone syndrome, but it can still grow or cause symptoms by its size or location. Ask which description applies to you. A positive somatostatin receptor scan does not by itself mean the tumour is functioning.

Sources: nccn-patient · nci-pnet

Your diagnosis record

Keep the primary site, differentiation, grade, Ki-67 if reported, stage or sites of spread, hormone syndrome and receptor-scan result together in your passport. Ask the team to explain changes rather than trying to interpret one result in isolation.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts measure infection-fighting cells, oxygen-carrying cells and platelets. Kidney and liver tests help the team assess how your body can handle treatment. The schedule for your medicine is shown separately. Attend these checks even when you feel well; a scan does not replace them.

Sources: nccn-patient · nccp

CT, MRI and receptor imaging

CT and MRI help show tumour size and where it is. A somatostatin receptor (SSTR) scan looks for receptor targets that certain treatments can attach to. An FDG-PET scan answers a different question about tumour activity and may be useful in selected cases. You will not necessarily need every scan. The specialist decides which test can change your care.

Sources: nccn-patient

Hormones and tumour markers are individual

The team may measure selected blood or urine markers when they answer a question about your NET or symptoms. These can include serotonin-related testing such as 5-HIAA for selected patients, or specific pancreatic hormones. Chromogranin A is not a universal test before every treatment. A changing marker alone does not prove cancer growth. Record why each test is being done, how to prepare and who will explain it.

Sources: nccn-patient · nci-net

Preparing for a test

Ask whether you need fasting, a timed urine collection, dietary preparation or a medicine review. Follow the laboratory’s instructions for your particular test. Do not stop an acid-suppressing medicine, diabetes medicine or hormone-control treatment yourself. Tell the team about kidney problems, allergies and all medicines before a scan or blood test.

Sources: nccn-patient · nci-net

Heart and inherited-risk checks

A heart ultrasound may be needed for particular treatment risks or to assess heart-valve problems associated with a hormone syndrome. It is not required on the same timetable for every NET. Depending on the diagnosis, age and family history, the team may offer genetic counselling; some NETs occur with inherited conditions such as MEN1. A tumour gene test and an inherited-risk test are not interchangeable.

Sources: nccn-patient

Who does what, and when?

Use the passport to write each test, its purpose, planned date, booking service, result-review contact and next step. Your consultant sets response-scan and marker intervals. The postoperative schedules in general guides are not automatically the right schedule while you are taking these medicines.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 06

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Record your usual bowel pattern, flushing, pain, appetite, weight and energy before starting. Note what changes, how long it lasts, what you were taking and how it affects daily life. This helps the team distinguish a hormone problem from treatment toxicity without assuming that every symptom is cancer progression.

Sources: nccn-patient

Hormone-related symptoms

Depending on the tumour, excess hormones may cause flushing, frequent watery stools, wheezing, acid-related pain or blood-sugar disturbances. An insulin-producing tumour can cause low blood sugar. Not every NET causes these problems. Ask which symptoms matter for your diagnosis and whether you need a separate rescue plan.

Sources: nccn-patient

Hormone treatment is a separate part of the plan

Somatostatin analogue injections and other symptom treatments may continue alongside anticancer treatment when prescribed. They may have their own appointment dates. Never stop them simply because tablets are paused. PRRT requires a carefully coordinated interruption and restart plan, which the nuclear-medicine service must supply.

Sources: nccn-patient · nccp

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Sources: nccp

Procedures and hormone syndromes

Tell the surgical, dental, anaesthetic or imaging team if you have a hormone-producing NET or a previous hormone crisis. Ask your specialist whether special preparation is needed. Carry your current medicines and emergency plan; the wallet card supports this information but does not replace specialist instructions.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 07

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Ask whether the main aim is to slow growth, reduce tumour size, relieve hormone symptoms, make another treatment possible or protect day-to-day quality of life. The aim and likely benefit depend on your particular NET and previous treatment. A stable scan may represent useful control even without tumour shrinkage.

Sources: nccn-patient

How treatments fit together

Some people need surgery, hormone-control treatment, tablets, chemotherapy, PRRT, liver-directed treatment or external radiotherapy at different points. These are not all automatically part of your regimen. Ask the multidisciplinary team to draw your own sequence and name the clinician coordinating it.

Sources: nccn-patient

Choosing between options

Ask why this option suits your tumour site, grade, growth rate, symptoms, receptor status and other health conditions. Discuss alternatives, the option of observation when appropriate, supportive care, and any suitable clinical trial. A treatment listed in a US guide is not a guarantee of availability or funding in Ireland.

Sources: nccn-patient

Evidence is about groups

A trial can measure time before cancer worsens, tumour shrinkage or survival. These are different outcomes. A group result does not predict your own response or lifespan. Ask how closely the study patients match your diagnosis and what benefit would make the treatment worthwhile to you.

Sources: nccn-patient

Review points and shared decisions

Agree when you will review symptoms, tests and scans, and what would lead to continuing, delaying, changing or stopping treatment. Discuss work, caring duties, travel and the effects you most want to avoid. You can revisit your choices as your situation changes.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Tell the team about weight loss, poor appetite, early fullness, persistent diarrhoea or greasy stools. Ask for a dietitian familiar with NETs and your surgery or medicines. Different causes need different support; do not assume every bowel symptom needs the same diet or medicine. Ask whether digestive-enzyme assessment or replacement is relevant to you.

Sources: nccn-patient · nci-pnet

Fluids and safe activity

Follow your individual fluid advice, especially with diarrhoea, kidney disease or heart problems. Balance manageable activity with rest. Tell the team if fatigue, dizziness, pain or breathlessness limits usual tasks; these symptoms can have treatable causes.

Sources: nccn-patient · nccp

Emotions, relationships and quality of life

Living with repeated tests and treatment can affect sleep, mood, relationships and confidence. Ask about psychological support, sexual health, fertility advice, symptom-control services and support for carers. Supportive or palliative care aims to improve comfort and daily life and can accompany active anticancer treatment.

Sources: nccn-patient

Practical help

Ask your nurse or social worker about transport, work, finances, home support, caring responsibilities and local NET support organisations. A general resource list may include services in other countries; your team can identify options available to you.

Sources: nccn-patient

A record that works without a phone

Keep the printed schedule, medicine list, tests, contact numbers and written emergency instructions together. The full handbook contains the information in sequence for offline reading. The QR code is an optional route to more information; it is not needed to call for help or follow your paper plan.

Sources: nccn-patient

Questions to take to the next visit

What is my exact diagnosis and grade? What is treatment trying to achieve? Which tests are mandatory for this medicine, and which depend on my NET? Who reviews results? What symptoms mean call now? What support would help me live as well as possible?

Sources: nccn-patient

Evidence for the pancreatic NET combination

ECOG-ACRIN E2211 compared CAPTEM with temozolomide alone.

Adding capecitabine improved the time before disease progression in the trial.

Sources: e2211

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of CAPTEM, and how was its off-label use explained?

Ask the team to explain the expected benefit, alternatives and burdens for your pancreatic NET. NCCP identifies this use as off-label and requires the local consent process.

Has my DPD result been reviewed?

DPD testing is required before first capecitabine unless already completed. Ask the team to confirm the result and any special plan; testing does not remove every risk of severe toxicity.

Can you write exact dose and meal times for Days 10–14?

Ask the pharmacist to write a practical calendar. Capecitabine is taken with food or within 30 minutes afterwards; temozolomide is taken fasting. Do not assume the two medicines should be swallowed together.

What anti-sickness medicines should I take on temozolomide days?

Temozolomide Days 10–14 has moderate-to-high oral emetic potential. Your team must prescribe the local preventive and rescue plan and explain its timing; this guide does not select a universal drug or dose.

When are liver tests after each cycle and after the last dose?

Liver tests are needed before and after each cycle. Injury may occur weeks after the final dose, so ask for the follow-up dates and whom to call for yellow skin/eyes or dark urine.

Which symptoms mean stop capecitabine and call?

Significant diarrhoea, mouth pain or hand-foot symptoms, vomiting that prevents drinking, fever or sudden illness needs prompt advice and capecitabine interruption as instructed. Chest pain or severe breathlessness needs emergency assessment.

When will we review the six-cycle plan and response?

The NCCP plan is six 28-day cycles unless progression or unacceptable toxicity changes it. Your consultant decides response-scan timing and whether the course should continue or change.

Try explaining the plan back: Can you point out the capecitabine days, the five temozolomide days, the different food instructions and your anti-sickness plan? What symptoms mean stop tablets and call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00505 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00505 v5 · current Irish CAPTEM regimen
- EMA Xeloda product information and patient leaflet
- EMA Temodal product information and patient leaflet
- NCCN Guidelines for Patients: Neuroendocrine Tumors, 2026 · education reference
- NCI PDQ: gastrointestinal NETs · selected marker background
- NCI PDQ: pancreatic NETs · hormone and digestive background
- ECOG-ACRIN E2211 randomised CAPTEM study

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6609/505_v5_Capecitabine_and_temozolomide__zFX2DSB.pdf

https://www.ema.europa.eu/en/documents/product-information/xeloda-epar-product-information_en.pdf

https://www.ema.europa.eu/en/documents/product-information/temodal-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/neuroendocrine-patient.pdf

https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq

https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq

https://pubmed.ncbi.nlm.nih.gov/36260828/